
VOTAZINE TABLET 25MG

DESCRIPTION: A 7 mm diameter, round, scored tablet, yellow in colour with marking 'd'.

COMPOSITION: Each tablet contains Promethazine Theoclate 25 mg.

PHARMACODYNAMICS: Antihistamine, antinauseant. It has a prolonged antihistamine action. Antihistamines diminish or abolish the main actions of histamine in the body, probably by occupying the receptor sites in the effector cells to the exclusion of histamine; they do not prevent the production of histamine. It is a histamine H₁-receptor antagonist; it has no effect on the stimulating action of histamine on the secretion of gastric acid. It is a phenothiazine derivative. Many histamine H₁-receptor antagonists have other pharmacological properties, largely independent of antihistamine activity, including anticholinergic, adrenaline – enhancing and/or adrenaline-antagonising and serotonin-antagonising effects. Some of these properties are not generally of beneficial clinical significance and may be regarded as side effects. Most histamine H₁-receptor antagonists cause depression of the central nervous system changing to stimulation with very large doses. Most have some local anaesthetic properties.

PHARMACOKINETICS: Promethazine is well absorbed after oral or intramuscular administration. Peak plasma concentrations have been observed 2 to 3 hours after administration by these routes, although there is low systemic bioavailability after oral administration, due to high first-pass metabolism in the liver. Promethazine is well distributed; it enters the brain, crosses the placenta, and passes into breast milk. Values ranging from 76 to 93% have been reported for plasma-protein binding. Promethazine undergoes extensive metabolism, predominantly to promethazine sulphoxide, and also to *N*-desmethylpromethazine. It is excreted slowly via the urine and bile, chiefly as metabolites. Elimination half-lives of 5 to 14 hours have been reported.

INDICATION: Although the primary indication for promethazine is the prevention and treatment of motion sickness, it has been used in the treatment and prevention of nausea and vomiting due to other causes, including: gastroenteritis and vestibular disturbance.

RECOMMENDED DOSAGE: Adults: Travel sickness: For prevention : Long journeys – One tablet each night at bedtime commencing on the night before traveling.

Short journeys – One tablet to be taken 1-2 hours before the journey commences.

For treatment – One tablet, followed by a second tablet the same evening and a third tablet on the following evening. Prompt treatment is important. Additional tablets may safely be taken as a preventive or when they appear to be needed but it will seldom be necessary to give more than four tablets in 24 hours or to repeat a dose in less than 8 hours.

Nausea and vomiting due to other causes: One 25 mg tablet repeated as required. Often one tablet at bedtime is sufficient. The maximum dose in 24 hours is 100 mg in divided doses.

Children: Travel sickness and nausea and vomiting due to other causes:

5-10 years – one half the corresponding adult dose.

Over 10 years – corresponding adult dose.

ROUTE OF ADMINISTRATION: Oral

CONTRACINDICATIONS: Promethazine should not be used in patients who are in a coma or suffering from CNS depression of any cause. It must not be given to neonates, premature infants or patients hypersensitive to phenothiazines. Promethazine should be avoided in patients who have been taking monoamine oxidase inhibitors within the previous 14 days. Promethazine is contraindicated in children less than 2 years old.

WARNINGS & PRECAUTIONS: Care is necessary with patients who take alcohol, anticholinergic agents, tricyclic antidepressants, sedatives or hypnotics, as such agents are enhanced by promethazine. It may cause mild and temporary confusion or disorientation in certain individuals. It should, therefore, be used with caution in persons in charge of vehicles until their reaction to the drug is known.

Promethazine may thicken or dry lung secretions and impair expectoration, it should therefore be used with caution in patients with asthma, bronchitis or bronchiectasis. Use with care in patients with severe coronary artery disease, narrow angle glaucoma, epilepsy or hepatic and renal insufficiency. Caution should be exercised in patients with bladder neck or pyloro-duodenal obstruction. Promethazine may mask the warning signs of ototoxicity caused by ototoxic drugs, eg. salicylates.

It may also delay the early diagnosis of intestinal obstruction or raised intracranial pressure through the suppression of vomiting.

Promethazine may interfere with immunologic urine pregnancy tests to produce false-positive or false-negative results. Promethazine should be discontinued at least 72 hours before the start of skin tests using allergen extracts as it may inhibit the cutaneous histamine response thus producing false-negative results.

In nausea and vomiting of unknown origin, it is essential to establish the diagnosis before giving an antiemetic, to ensure that a serious underlying condition is not masked.

Promethazine lowers seizure threshold. It should also be used with caution in the following conditions: respiratory depression, bone marrow depression and neuroleptic malignant syndrome.

Use in children: This product contains promethazine theoclate. It should not be used in pediatric patients less than 2 years of age because of the potential for fatal respiratory depression. Caution should be exercised when used in children 2 years of age and older.

INTERACTION WITH OTHER MEDICAMENTS: Interactions of Antihistamines: Sedating antihistamines may enhance the sedative effects of CNS depressants including alcohol, barbiturates, hypnotics, opioid analgesics, anxiolytic sedatives, and antipsychotics. Sedative interactions apply to a lesser extent with the non-sedating antihistamines; they do not appear to potentiate the effects of alcohol, but it should be avoided in excess. Sedating antihistamines have an additive antimuscarinic action with other antimuscarinic drugs, such as atropine and some antidepressants (both tricyclics and MAOIs). It has been suggested that some sedating antihistamines could mask the warning signs of damage caused by ototoxic drugs such as aminoglycoside antibiotics. Antihistamines may suppress the cutaneous histamine response to allergen extracts and should be stopped several days before skin testing.

USE IN PREGNANCY & LACTATION: When given in high doses during late pregnancy, phenothiazines have caused prolonged extrapyramidal disturbances in the child. Safety in pregnancy has not been established. To be used if benefit outweighs risk.

SIDE EFFECTS:

More common reactions:

Gastrointestinal: Dry mouth, epigastric distress, loss of appetite, nausea, vomiting, diarrhoea, constipation.

Nervous system: Sedation, restlessness, dizziness, lassitude, incoordination, fatigue.

Ocular: Blurred vision.

Less common reactions:

Cardiovascular: Tachycardia, bradycardia, faintness, palpitations, hypotension, arrhythmias.

Dermatological: Contact dermatitis (topical), photosensitization, urticaria, angioneurotic oedema.

Haematological: Leucopenia, agranulocytosis, aplastic anaemia, thrombocytopenia purpura.

Hepatic: Jaundice.

Musculoskeletal: Extrapyramidal symptoms.

Nervous system: Tinnitus, euphoria, nervousness, insomnia, convulsive seizures, oculogyric crises, excitation, catatonic-like states, hysteria, extrapyramidal symptoms, tardive dyskinesia.

Respiratory: Marked irregular respiration.

Other: Anaphylaxis, headaches, nightmares, urinary retention.

Serious or life threatening reactions: Agranulocytosis.

Care is needed in the intramuscular administration of promethazine to children. A severe neurological reaction resulting in coma is possible.

SYMPTOMS AND TREATMENT OF OVERDOSE: Symptoms: The chief symptom of acute poisoning from the ingestion of promethazine is unconsciousness which is, however, commonly delayed. In addition, convulsions have occurred, with unconsciousness in the intervening periods.

Treatment: An immediate first aid measure is to induce vomiting mechanically or to give an emetic, the value of which, however, is limited by the antiemetic activity of promethazine once absorbed. The most important step in treatment must be, therefore, to remove as much as possible of the unabsorbed material by means of gastric lavage with warm sodium bicarbonate solution. Some sodium bicarbonate solution should be left in the stomach to precipitate insoluble promethazine base, thus delaying its absorption. Should convulsions occur, special care must be taken of the use of sedatives which may increase the depression of respiration. Intravenous amphetamine or intramuscular ephedrine should be given for CNS stimulation, if indicated. Oxygen under pressure should be given if respiration is depressed. The administration of an antibiotic as a prophylactic against pneumonia should be considered.

STORAGE CONDITIONS: Store below 30°C. Protect from light. Keep out of reach of children. *Jauhkan daripada kanak-kanak.*

SHELF LIFE: Please refer to the outer package.

PACK SIZE: Pack of 1000 tablets in loose (FOR EXPORT ONLY) and blister pack.

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD (42491-M)

Lot 2599, Jalan Seruling 59 Kawasan 3,

Taman Klang Jaya, 41200 Klang, Selangor, MALAYSIA.